COUNTRY PROFILE AND MARKET INTELLIGENCE

Singapore

Executive summary



AI-generated depiction of Marina Bay Sands and the ArtScience Museum; not a photograph of the site.

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Prepared 7 October 2026. Analyst review draft. Numbered citations refer to Singapore_Profile_DRAFT_SOURCES.docx.

At a glance

6.21M Total population, June 2026[3]	**19.5%** Residents aged 65+, June 2026[7]
83.9 years Resident life expectancy at birth, 2025[7]	**Int$163,354** GDP per capita, PPP, 2025[5]
5.0% Real GDP growth, 2025[12]	**US$3,922** Current health spending per person, 2023[26]
2.9 Registered doctors per 1,000 total population, 2024[30]	**10,784** Public acute hospital beds, 2025[19]

Country background

Singapore is a city-state and parliamentary democratic republic, with Singapore as its capital.[1] Prime Minister Lawrence Wong heads the government and holds the finance portfolio; President Tharman Shanmugaratnam is the non-partisan head of state.[8,9] English, Malay, Mandarin and Tamil are official languages.[2] The economy generated S$789.5 billion in nominal GDP in 2025.[12] Its Economy 2030 strategy emphasizes advanced manufacturing, skills and technology adoption.[14] In August 2026, the government raised its full-year growth forecast to 4.5% to 5.5%, citing stronger global investment related to artificial intelligence.[13]

Of 6.21 million people in June 2026, 4.23 million were residents, comprising citizens and permanent residents.[3] Aging is a central healthcare demand driver: people aged 65 and older represented 19.5% of residents and 21.4% of citizens.[3,7] The resident fertility rate fell to 0.87 births per woman in 2025.[7] Cancer, pneumonia and ischemic heart disease led recorded mortality in 2025, while national policy increasingly emphasizes prevention, community support and coordinated care.[15,21,36]

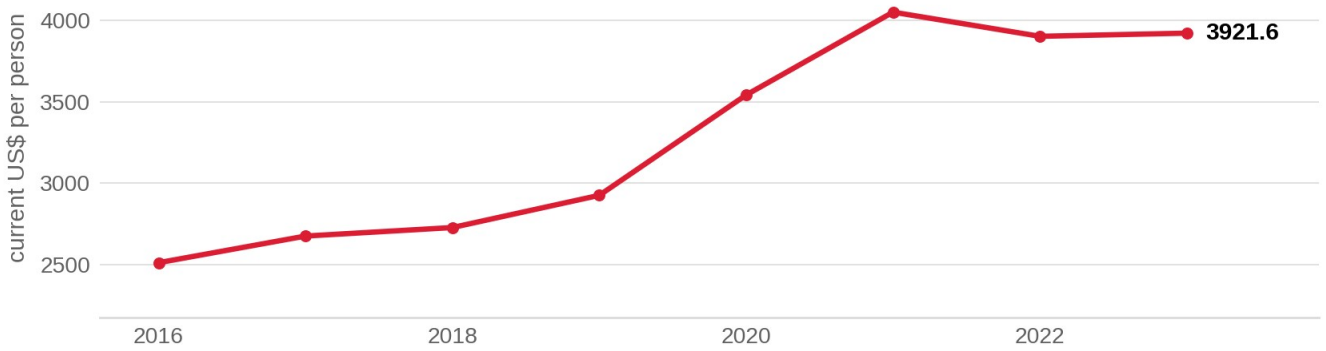
Key takeaways for CSI

- Three public healthcare clusters combine service delivery with regional population-health responsibilities.[11]
- Healthier SG connects residents aged 40 and older with regular family doctors and prevention plans.[21]
- MOH plans approximately 2,800 additional public acute and community beds by 2030.[20]
- International academic relationships span medical education, cardiovascular research and specialist knowledge exchange.[38,39]

Health system

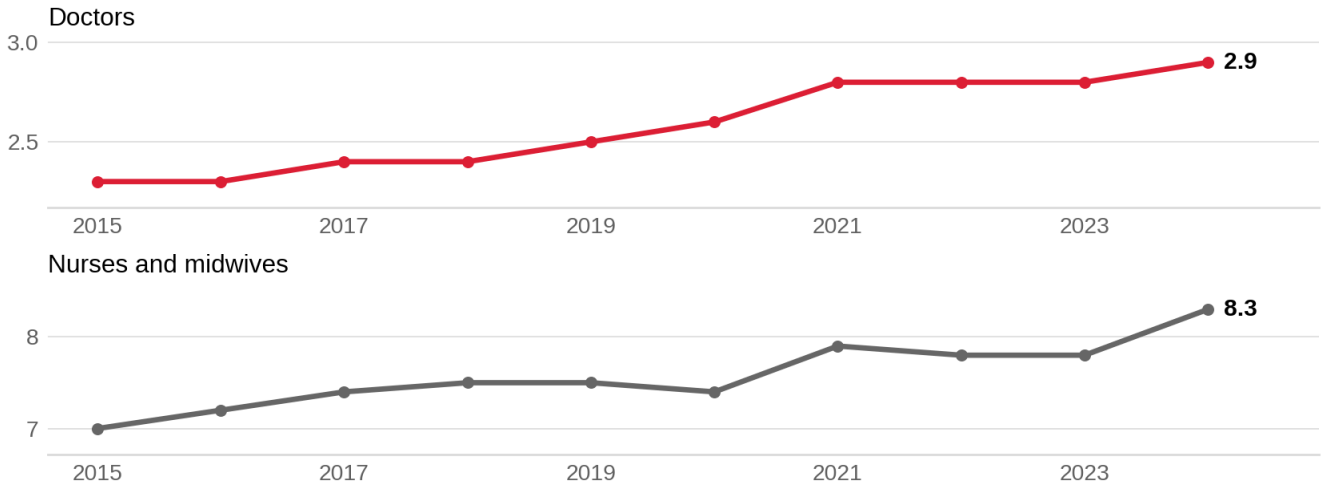
- Financing combines public subsidies, compulsory medical savings, national catastrophic insurance and means-tested assistance.[22]
- MediShield Life provides lifelong coverage for citizens and permanent residents.[29]
- Registered workforce ratios include inactive practitioners and differ from OECD practicing-workforce measures.[28,30]

Health spending per person, 2016 to 2023



Current health expenditure; nominal US dollars, not inflation-adjusted or PPP-adjusted. Sources: [26]

Health workforce density, 2015 to 2024

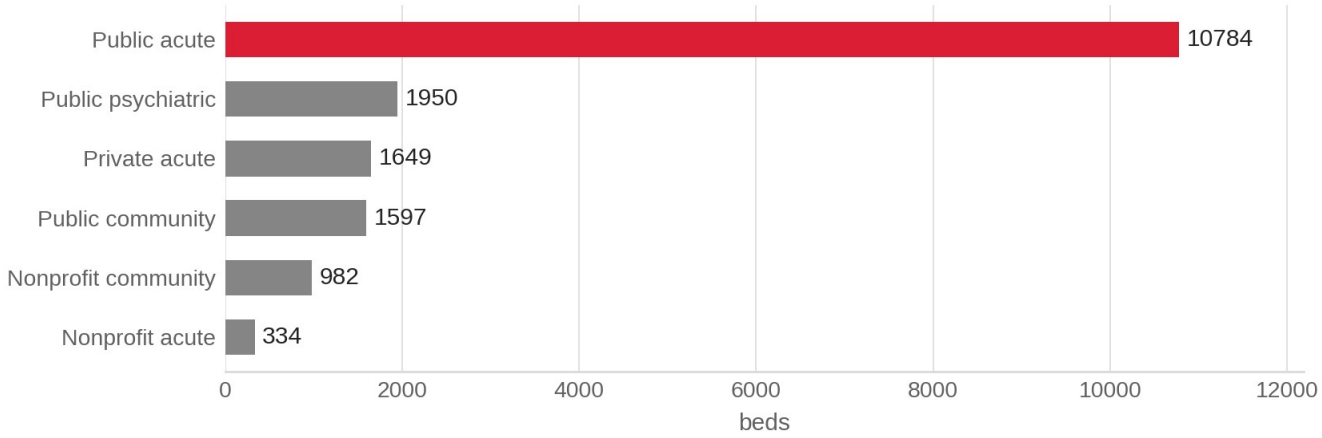


MOH/SingStat registration-based series; nurses include registered and enrolled nurses and midwives. Published per-10,000 rates divided by 10. Sources: [74]

Market and competition

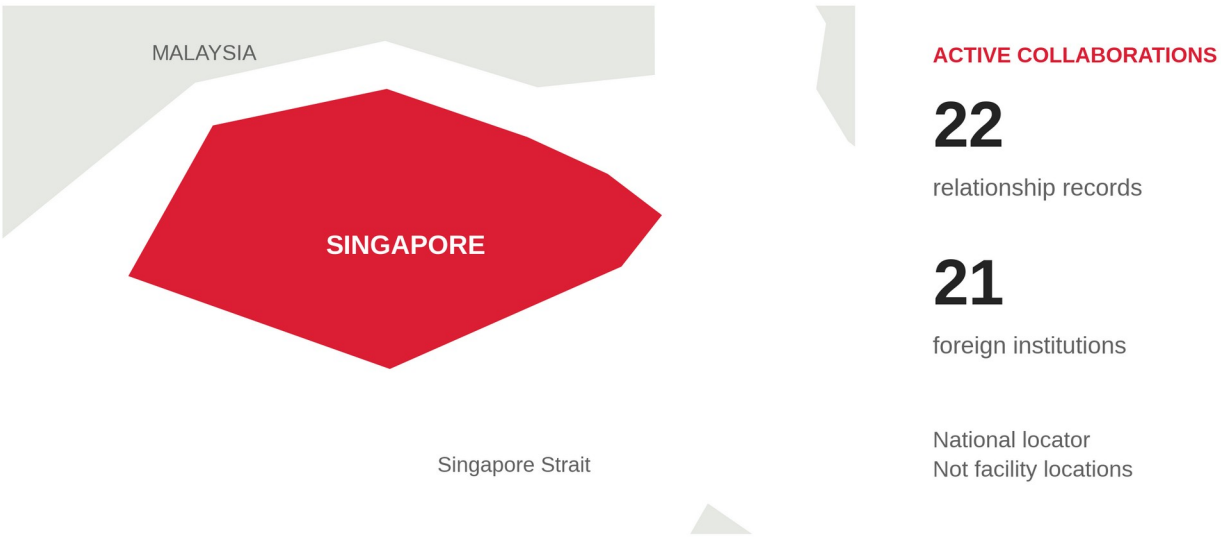
- Eight private acute hospitals provided 1,649 beds in 2025.[18,19]
- Private primary care included 2,777 general-practitioner clinics in 2025.[18]
- Raffles Hospital supports international patients through clinical appointments and non-medical assistance.[24]

Hospital bed capacity by ownership and care setting, 2025



Mutually exclusive hospital-bed categories; nursing-home beds are excluded. Sources: [19]

International institutional collaboration



Active external institutional collaborations, reviewed 7 October 2026. Directory totals exclude Cedars-Sinai. Map: Natural Earth, public domain. Counts describe relationships in the country, not hospital ownership or facility locations.

Institution directory

21 foreign institutions with dated evidence of active collaboration. Entries 1 to 21. Detailed evidence appears in the profile and tracker.

Institution	Nature of collaboration
Duke University School of Medicine	Research collaboration, Training and education
Harvard University (Harvard T.H. Chan School of Public Health)	Research collaboration, Training and education
Johns Hopkins Health System	Training and education
Mayo Clinic	Research collaboration
Methodist Le Bonheur Healthcare	Training and education
Michigan Medicine (University of Michigan)	Research collaboration, Training and education
Mount Sinai Health System	Training and education
NYC Health + Hospitals	Training and education
Northwell Health	Training and education
Stanford Health Care	Training and education
Stanford Medicine	Research collaboration, Training and education
Beijing Tsinghua Changgung Hospital	Training and education
CUHK Medical Centre	Training and education
Imperial College London	Training and education
Karolinska University Hospital	Training and education
National Cancer Center Japan	Research collaboration
Ruijin Hospital (Shanghai Jiao Tong University School of Medicine)	Research collaboration
Shanghai Jiao Tong University School of Medicine	Training and education
University Health Network	Training and education
University of Cambridge School of Clinical Medicine	Research exchange through joint symposia
University of Oxford (Oxford University Clinical Research Unit)	Clinical-trial capacity development and training

CSI position and risks

CSI partnerships in Singapore

[MANUAL INPUT REQUIRED]

To be completed by CSI from internal data and relationship assessment.

Opportunities for CSI

[MANUAL INPUT REQUIRED]

To be completed by CSI from internal data and relationship assessment.

Risks and watch items

- More than half of hypertensive residents aged 18 to 74 examined in 2023 to 2024 were previously undiagnosed.[16]
- The healthcare workforce is projected to rise from 129,000 in 2024 to approximately 156,000 by 2030.[31]
- Health-information contribution requirements phase in from September 2027 and September 2028.[37]

Recent developments

2026-10-03

NPHS 2025 reported 87.3% of residents aged 18 to 74 met the sufficient-physical-activity definition.[17]

2026-09-09

MOH announced regulatory consolidation under HSA, with transfers scheduled for November 2026 and by end-2027.[72]

2026-05-06

MOH outlined phased health-record contributions following February 2026 enactment of the Health Information Act.[37,73]